

ORIGINAL ARTICLE

Perceptions of Educational Community Members: Deployment of a University Health Promotion Policy

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ABSTRACT

Universities have been the focus of numerous studies on health promotion initiatives, mainly centred on results that determine outcomes and impacts. This study explores the perceptions of various community members regarding implementing the university health promotion policy. We employed qualitative methods (case study); semi-structured in-depth interviews were conducted at a Chilean university to gather data based on both inductive and deductive analyses. A comprehensive schema of students', professors' and administrative/technical staff's perceptions was established. We found a generally positive assessment of the policy and its promotion of healthy lifestyle environments. However, participation within the university community was uneven, with each group identifying different barriers to increased involvement: high academic workloads (students), research and administrative responsibilities (professors), and limited availability of health and wellness-promoting activities (administrative/technical staff). The findings suggest that future educational health initiatives should incorporate health indicators into management practices, enhance wellness literacy and empower community members to foster greater engagement. From a methodology perspective, study design enables the replication of health policy or program deployment in educational community health settings.

1 | Introduction

Health-promoting settings have been positioned at the centre of debates among scientists and decision-makers in health policy. Thus, the objectives of achieving healthy communities and wellness societies have been addressed through health promotion, ensuring that individuals and communities can take control of their health and experience an entire life by addressing the determinants of health in all settings (World Health Organization 2021). Ecological and social determinant models based on the interactions and influences of economic, political, cultural, environmental and

individual factors that determine health and well-being have been proposed to achieve health-promoting settings where people and communities live (World Health Organization 1986). UNESCO has urged higher education institutions to play a more active role in the social transformations necessary to fulfil the SDGs, where health and well-being have emerged as fundamental (Hessen and Schmelkes 2022).

The response of universities has been the creation of health and wellness promotion settings where the daily occupational and student lives of many people take place for extended periods

(Suárez-Reyes and Van DenBroucke 2016). In these critical health and wellness promotion environments, students live in a stage of change and experimentation, whereas the administrative/technical staff and professors have been subjected to organisational policies; in both groups, health and wellness can deteriorate, given certain practices. Among the main problems universities face are the decline in mental health and its detrimental impact on students' academic performance (Olsson et al. 2024), as well as sedentary working conditions for university staff (Hanna et al. 2019). Therefore, implementing university health promotion policies addresses organisational structure and culture, positively affecting the entire community rather than just specific target groups' activities (Olsson et al. 2024). Consequently, decision-makers in scientific (Dooris and Doherty 2010) and health policy (Okanagan Charter 2015) spheres recommend implementing health and wellness policies in universities and integrating them into their functions, organisational levels and strategic plans. In Iberoamerica, the health-promoting universities' movement has strengthened the health of university communities since 2003 through the implementation of health promotion programs (Arroyo-Acevedo et al. 2014).

Health promotion policies in universities as an instrument of institutional management have been approached from three perspectives. First, they aim to create support, organisation, culture and learning in working and living settings. Second, these policies integrate health into the university's organisational environment, focusing on the entire university community (Dooris et al. 2019). Lastly, they aim to positively impact the health and well-being of communities through education, research and institutional practices (Dooris et al. 2012; Olsson et al. 2024). However, evaluation initiatives in health promotion policies have not been sufficiently systematic to provide inputs to decision-makers (Dooris 2001; Duarte Cuervo 2016). Although measurements have been conducted on mental health (Saheb et al. 2019), physical exercise, smoking, sedentary lifestyle (Venturoso Gongora Buckeridge Serra et al. 2023) and sleep hygiene (Ahlstrand et al. 2022), which generate significant contributions, the evaluations have not been extended to a comprehensive view of health promotion and well-being (Saheb et al. 2019).

The challenge of obtaining information on the results of the implementation of health promotion policies in higher education institutions has been addressed by various authors. These investigations have been carried out in university populations from quantitative (El Ansari et al. 2011), qualitative (Dooris et al. 2021), mixed (Suárez-Reyes and Van DenBroucke 2023) and conceptual (Darker et al. 2023) perspectives. Studies in this field have focused predominantly on the student population (Bachert et al. 2021; Skoglund et al. 2021); therefore, it is necessary to delve deeper into the perceptions of the three groups that make up the university community: students, administrative/technical staff and professors (Suárez-Reyes and Van DenBroucke 2023). Thus, qualitative research and a systemic vision have been conducted to assess the perceptions of the different actors that make up the university community (Ferguson 2011; Ferreira et al. 2019), addressing the intricate contextual relationships in health-promoting universities (Holt et al. 2015; Reis et al. 2018).

This study was conducted as a case study at the Universidad Católica del Norte, Coquimbo, Chile. This university health

promotion policy was first enacted in Chile and was one of the first enacted in Iberoamerica. The objectives were as follows: (i) to explore the perceptions of the different actors of the educational community regarding the university health promotion policy from past, current and future perspectives, and (ii) to assess the perceptions of the different actors of the university community to implement improvements in the university health promotion policy.

2 | Methods

An exploratory descriptive qualitative study was conducted under an interpretive paradigm to address the case study (Creswell and Poth 2018; Miles et al. 2014; Yin 2018) at Universidad Católica del Norte, Coquimbo, Chile (UCN). This had as its central characteristic the analysis of a unit's complexity (Stake 1998) and sought the understanding of the phenomenon. Thus, the analysis focused on two areas of interest: exploring and assessing perceptions regarding various key actors within the UCN educational community. This analysis is relevant within the case study context, as it will consider past, present and future perspectives to implement improvements in the university policy for promoting health and quality of life at the UCN (henceforth VIVE UCN policy). Finally, the study was of a synchronic type because it was carried out only once and in a specific period. Follow-up of the evolution of the study participants' perceptions was not considered.

2.1 | University Health Promotion Policy VIVE UCN

The VIVE UCN policy was declared in 2012 due to a co-constructed creation process with the participation of students, professors and administrative/technical staff. The policy declared three strategic lines aimed at the well-being of the university community and the generation of healthy settings. First, it promoted healthy settings and initiatives on campus. Second, it contributed to forming integral professionals capable of responsibly managing their health and that of their settings in a responsible manner. Finally, it contributed to the quality of occupational life for the professors and administrative/technical staff.

For the VIVE UCN policy implementation, an executive committee composed of the university community developed action plans, evaluation instances and improvement proposals. The actions proposed by the executive committee resulted from the conjunction of needs raised by the university community and the mandates of creating healthy settings declared in the university's health promotion policy. These actions were carried out during a healthy pause called ENCUÉNTRATE (every Wednesday between 10:00 and 11:30 am). During this pause, the entire university community was invited to develop healthy community or self-care activities in the university's settings, whether created (i.e., healthy settings) or in the natural assets of the university campus.

2.2 | Participants

The study population comprised members of the UCN community, specifically students, professors and administrative/

technical staff. The qualitative method was employed to explore the subject, and purposive sampling was utilised (Patton 2002) with various educational stakeholders (Prestigiacomo et al. 2024) due to the diverse wealth of information and the opportunity for sampling based on convenience criteria. The inclusion criteria were belonging to the UCN community, past or current participation in the VIVE UCN policy and gender parity.

The sample consisted of 15 key stakeholders ($n=15$) involved in the VIVE UCN policy (Table 1). The sample was distributed by gender: 40% male and 60% female. The average age was 41.87 years (SD = 12.91). All interviewees had a minimum of five years' tenure at UCN.

2.3 | Data Collection and Analysis

Data were gathered through individual interviews with key informants from May to June 2024. Semi-structured interviews were conducted (Howitt 2019) and lasted an average of 50 min. The interview schedule was organised into four thematic sections. The first section included general information questions (e.g., What is your role in the educational community?). The second section included questions about concepts (e.g., Do you know the health promotion policy VIVE UCN? What can you tell me about it?). The third section evaluated the VIVE UCN policy (e.g., What facilitators or obstacles have you perceived in implementing this policy?). The fourth section comprised evaluation/concluding remarks (e.g., What is your overall assessment of the VIVE UCN policy?). The registration process involved audio recording and subsequent verbatim digital transcription.

Inductive and deductive analyses were conducted with the support of the ATLAS.ti 24 software. As proposed by Mayring (2022), thematic content analysis was used from a descriptive perspective, following the stages of pre-analysis, definition of units, elaboration of codes (deductive and inductive), definition of categories and a comprehensive synthesis. The

coding process resulted in 43 codes (Figure 1), grouped into eight categories (Table 2) and 15 memos. In addition, a documentary analysis was carried out to interpret the data collected and support the preparation of the final report. For documentary analysis, internal and external press releases, official university social networks and minutes of meetings with VIVE UCN policy executors were reviewed.

Following the process of analysis and interpretation of the information, the codes and categories were reviewed, which led to the establishment of five significant segments of analysis: (1) assessment of the VIVE UCN policy; (2) past, present and future VIVE UCN policy; (3) VIVE UCN: university health promotion policy or instrument? (4) time for health and well-being or academic and productive success? and (5) different advices from community members on how to improve the VIVE UCN policy. These segments allowed us to comprehensively address the core elements that guided the study: perceptions, evaluation, perspectives, challenges and improvements to the policy, allowing an understanding of the case study. The Results section presents the findings organised according to these five defined segments.

2.4 | Ethical Considerations

This study only included voluntary participants who were declared willing to participate by signing an individual informed consent document. This study followed the ethical guidelines of the American Psychological Association (APA) and was approved by the Scientific Ethical Committee of the UCN (CECFAMED-UCN Rev. N°11/2024).

The scientific rigour criteria included transferability, dependability and credibility. For the latter, interviews and preliminary analyses were conducted by an external social science professional collaborator who had no prior involvement in or knowledge of the VIVE UCN policy. The researchers participated in the methodological design and sample definition, and were re-incorporated for formal analysis using the researcher triangulation procedure (Flick 2015) to minimise biases.

TABLE 1 | Sample distribution.

	Students	Administrative/ technical staff	Professors
<i>n</i>	5	5	5
%	33.3	33.3	33.3

3 | Results

Through qualitative analysis, the interpretations of the information showed the appropriation of the university health

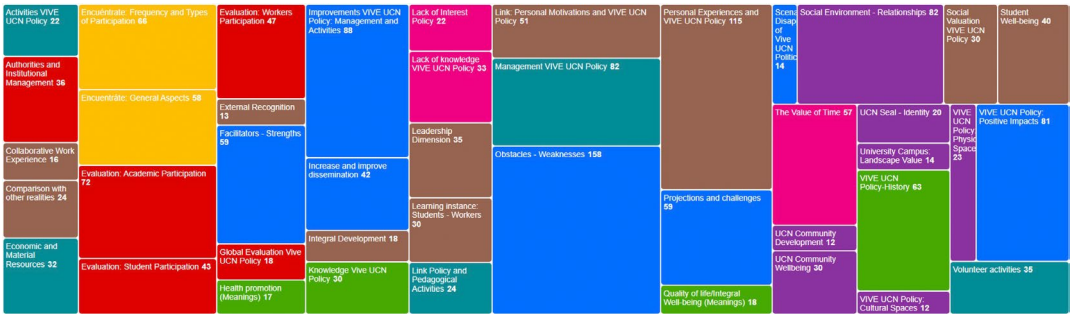


FIGURE 1 | Grouping of total codes by categories using a Word Tree Map (elaborated with ATLAS.ti software).

TABLE 2 | General overview of the categories and code distribution.

Group (categories)	Codes
‘ENCUÉNTRATE UCN’	Frequency and types of participation
	General aspects
Achievements and advances	Collaborative work experience
	Comparison with other realities
Obstacles to progress	Lack of interest policy
	Lack of knowledge VIVE UCN policy
	The value of time
Knowledge and meanings	Health promotion (meanings)
	Knowledge VIVE UCN policy
	Quality of life/integral well-being (meanings)
	VIVE UCN policy—history
Management and activities	Activities VIVE UCN policy
	Economic and material resources
	Volunteer activities
	Link policy and pedagogical activities
	Management VIVE UCN policy
VIVE UCN evaluation	Student participation
	Professor’s participation
	Administrative/technical staff participation
	Authorities and institutional management
	Global evaluation VIVE UCN policy
University community development	Social environment—relationships
	UCN community development
	UCN community well-being
	UCN seal—identity
	University campus: landscape value
	VIVE UCN policy: cultural spaces
	VIVE UCN policy: physical space
Achievements and advances	External recognition
	Leadership dimension
	Learning instance: students—workers
	Social valuation VIVE UCN policy
	Student well-being
	Personal experiences and VIVE UCN policy
	Integral development
	Link: personal motivations and VIVE UCN policy

(Continues)

TABLE 2 | (Continued)

Group (categories)	Codes
VIVE UCN policy: present and future	Improvements VIVE UCN policy: management and activities Increase and improve dissemination Facilitators—strengths Obstacles—weaknesses Projections and challenges Scenario: disappearance of VIVE UCN politics VIVE UCN policy: positive impacts

promotion policy in the UCN (henceforth VIVE UCN policy) and the healthy pause promoted by the VIVE UCN policy (henceforth ENCUÉNTRATE). The results were organised into (1) assessment of the VIVE UCN policy; (2) past, present and future VIVE UCN policy; (3) VIVE UCN policy: health promotion university policy or instrument; (4) time for health and well-being or academic and productive success and (5) different advices from community members on how to improve the VIVE UCN policy.

3.1 | Assessment of the VIVE UCN Policy

The university community had a positive perception of the VIVE UCN policy due to the generation of positive impacts on participants over time. However, the perception of the policy differed according to the university community (i.e., students, professors and administrative/technical staff) and their level of past or present involvement in the policy. The materialisation of the valuation was seen in the connection of their life experiences with aspects such as the acquisition of healthy lifestyles, promotion of a sense of community, integral formation and generation of feelings of identity, which emerged when compared to other universities or other occupational settings.

[I] wanted to be better and feel better, to be healthier, to exercise more ... in other words, I grew as a person in the spaces of the VIVE UCN policy.

(Student)

[and] we meet as peers: students, professors, and administrative/technical staff. So, you find yourself in another setting ... and we are called to do some activity in which we have common healthy interests.

(Professor)

The construction of collective well-being was closely related to the social climate and the relationships between university members. The latter were emergent findings not stated in the VIVE UCN policy. The perception of the university community emerged from their experience in implementing actions of the VIVE UCN policy, such as the creation of healthy settings (e.g.,

hammocks for relaxation) or in the social bonding and affiliation groups (e.g., the runners' group), which were enhanced by the natural setting of the campus.

We have a privileged space [referring to the natural assets of the campus], for sports activities, culture, etc. ... all beneficial for students at a university that are being formed in an integral space.

(Student)

Everyone brings their things, they gather the tables and prepare breakfast together ... it is like a minute of trust where you stop being hierarchical to know how your staff is, so you get to know people better.

(Administrative/technical staff)

The actors of the university community valued the explicit declaration that the university was concerned about the well-being. The VIVE UCN policy obliged the institution to implement it but also demanded that each actor of the university community embrace the challenge of being protagonists in leading healthy actions in this crucial setting and their lifestyles.

It is good to work in an institution that does care [about people's health] ... So, what has changed in me? I think the constant concern for valuing being healthy, healthy life, healthy settings

(Administrative/technical staff)

3.2 | Past, Present and Future VIVE UCN Policy

The interviewees' perceptions regarding the achievements of the VIVE UCN policy made evident gradual progress over time. The records of the historical memory of the members of the community showed active participation in the creation of the policy (past), a scenario of strengthening (present) and challenges to project its consolidation (future). Thus, the trajectory of the VIVE UCN policy made it possible to theorise on its implementation stages: declaration, action and consolidation.

I intervened in the process in which the participatory process to generate the policy as VIVE UCN policy was beginning.

(Student)

Today, after the pandemic, people have seen an exponential appreciation [VIVE UCN policy].

(Administrative/technical staff)

The institution has matured over the past decade ... [now] it's time for the individuals themselves to reach a stage of maturity.

(Professor)

The implementation of the VIVE UCN policy faced challenges due to a lack of understanding and irregular participation from the university community. Effective dissemination activities and ensuring everyone is responsible for creating a healthy setting are essential to address these issues.

[a]lso we have a lot of new administrative/technical staff who do not know about this [VIVE UCN policy].

(Administrative/technical staff)

Although the actors of the university health promotion policy showed appropriation of the policy and healthy pauses, the dissemination activities were understood as the responsibility of the institution and generated a disconnection with their responsibility. Some actors of the university community dissociated themselves from the leading role they should play in a healthy, co-constructed setting and delegated these responsibilities to other actors.

I would give it a low grade in diffusion, mainly the diffusion of the ENCÚENTRATE (healthy pause) among the student body. Some say 'Professor, what is ENCÚENTRATE?'....

(Professor)

3.3 | VIVE UCN: University Health Promotion Policy or Instrument?

The university health promotion policy (VIVE UCN policy) required instruments like ENCÚENTRATE, where the entire university community was called upon to organise or participate in healthy activities. However, high participation in ENCÚENTRATE overshadowed VIVE UCN policy; hence, both were understood by the university community as equivalent, unable to establish differences between them.

[o]ne of the issues that we have tried to socialize is that the policy is not only the ENCÚENTRATE

(Student)

[I]n fact, when we have gone to induction, 'What is the VIVE UCN policy? [they ask you]', 'The ENCÚENTRATE [some answer]'.

(Administrative/technical staff)

3.4 | Time for Health and Well-Being or Academic and Productive Success?

An emerging finding was the dedication that different interviewees gave to their time. Some students used the healthy pause for activities that promoted well-being, whereas others dedicated it to academic subjects. This healthy pause was presented as the appropriate space for students engaged in volunteer activities, sports activities and community health promotion activities.

I, until the third year of university, occupied the ENCÚENTRATE to study. The fourth year, I said no, that's enough ... it was too much academic workload, bad sleep, and stress.

(Student)

For the administrative/technical staff, participation and involvement in the space subscribed to the execution of various initiatives during a healthy pause. Other uses appreciated were dedicating this time to relaxation, leisure or social meetings. Participants were either active (participating in healthy pauses) or passive (resting without participation). In addition, the improper use of healthy pauses was noted, and although punctual, peers highlighted them. Finally, interviewees indicated fewer activities for administrative/technical staff than for students.

Because there are instances when I have gone running inside here, but I spend more time in the gym [both ENCÚENTRATE activities], supporting the students and co-workers there

(Administrative/technical staff)

There are a lot of people who go out for personal transactions [outside the university]. Imagine that some of those people have an accident ... that would be the end of the ENCÚENTRATE.

(Administrative/technical staff)

The professors showed a neutral perception of the healthy pauses. Thus, the interviewees perceived the low participation of professors. Despite this generality, there were experiences of healthy use of the pause in non-work activities, such as sports and recreation.

We professors see it differently because we are inserted in a competitive system, and teaching,

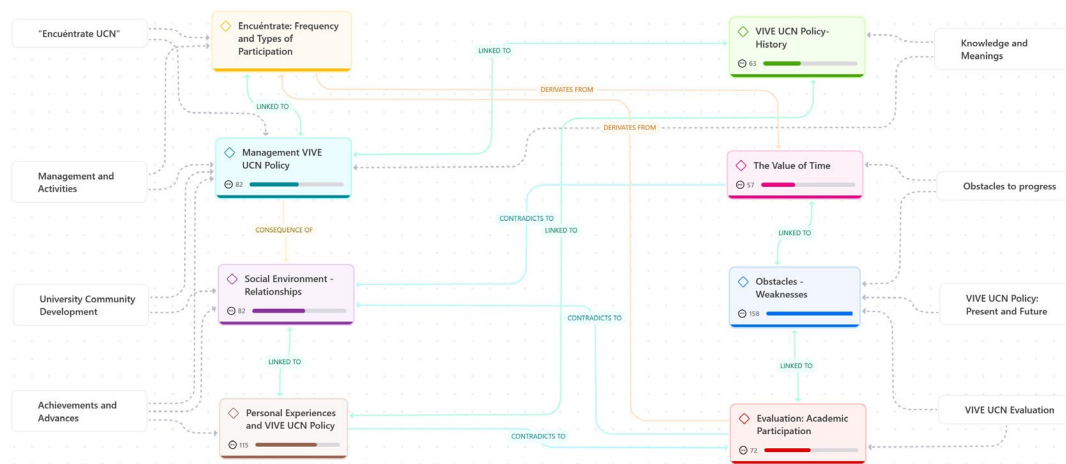


FIGURE 2 | A comprehensive diagram linking the perception of VIVE UCN policy.

research, and publication indicators measure us. So, we spend our time meeting those indicators.

(Professor)

There should be a review process, I do not know, every two years, where we are invited to discuss and evaluate some improvements [in VICE UNC policy].

(Administrative/technical staff)

3.5 | Different Advices From Community Members on How to Improve the VIVE UCN Policy

The participation of students, professors and administrative/technical staff in the policy deployment made it possible to build different experiences around a health-promoting university. These experiences allowed the interviewees to outline diverse visions of how the VIVE UCN policy should be projected in the face of a consolidation stage.

From the student's perspective, the need to increase participation emerged, that is, to involve the different actors of the university community to promote virtuous connections and co-existence in healthy settings.

[I]acking the participation of the professors [as leaders] ... to involve, motivate, and disseminate about the activities carried out.

(Student)

Among the administrative/technical staff, there was participation in the VIVE UCN policy and the healthy pause, which evidenced the need to inquire about the participation of new entrants to the university. They also highlighted the appropriation of the physical and social spaces represented by the ENCUÉNTRATE and that, given the institutional nature of the VIVE UCN policy, it should be safeguarded so that there are no labour or organisational barriers that hinder their participation.

I do not know if there has been an evaluation of how many of the new administrative/technical staff are participating in the VIVE UCN policy.

(Administrative/technical staff)

I tell my colleagues, this is something institutional, that is, you must adhere to the policies, 'Ah, it's just that I'm not interested' [they respond]. That's a pity, that's a pity

(Professor)

[self-care] has to do with a life option or a lifestyle and finding it in your workplace gives you a lot of satisfaction because it allows you to give coherence and continuity to what you do daily.

(Professor)

Finally, the VIVE UCN policy must face the challenge of moving towards its consolidation stage. Empowering the university community and institutional recognition as a strategic organisational asset must be key elements.

I believe that [institutional] strategies can be established ... to socialize and communicate better what we do, to generate synergies [between different units of the university].

(Professor)

Figure 2 shows the global analysis of the VIVE UCN policy. This presents a comprehensive scheme of the factors linked to its evaluation. Different related elements within the temporal and subjective construction framework mediated the VIVE UCN policy. Figure 2 also presents the interactions that made it possible to advance the understanding of the policy, according to the eight categories detected, together with the code that grouped the most significant number of references in the text. Thus, the following were highlighted: the linkages—Linked to—that occurred between codes (e.g., VIVE UCN policy history and VIVE UCN policy management), —Contradicts to—opposed codes (e.g., social environment—relationships and the value of time), codes derived from each other—Derivates from—(ENCUÉNTRATE: frequency and type of participation, and evaluation: academic participation) and others that were a consequence—consequence of—(e.g., VIVE UCN policy and social environment relationships).

4 | Discussion

The results of this qualitative study allowed us to advance the understanding of the case of a university health promotion policy in a Chilean context. Thus, from a vision built over time, university community members showed a positive perception of the policy's creation, implementation and current performance. All of this is nuanced by the healthy setting promoted by the policy and natural assets of the university campus. The university health promotion policy (VIVE UCN) and natural capital allowed students, administrative/technical staff and professors to experience the policy's activities, such as the healthy pause (ENCUÉNTRATE). These experiences constituted the crucial factor for the positive valuation of the policy and also promoted and strengthened the commitment to the health and well-being of the actors (Farrier et al. 2017). A sample of this web of interactions that give life to the VIVE UCN is shown in the comprehensive scheme (Figure 2), which provides a global overview of the case study.

One finding was related to the trajectory of the VIVE UCN policy enacted in 2012; the perception of the university community highlighted participatory construction in its beginnings, a current scenario with challenges for its strengthening and a future that must be projected towards consolidation. The extension in time of the VIVE UCN policy also evidenced obstacles due to the succession periods. According to Bano et al. (Bano et al. 2022), the succession of actors affects organisational performance due to the loss of policy consistency of the renewal of institutional actors. For the interviewees, succession was a problem, given the rotation of students (due to graduation) and the renewal of professors and administrative/technical staff (due to retirement and new hires). Thus, this loss of human capital must be addressed by effective succession plans (Rothwell 2010) that consider instances of induction and recruitment of new talent to give continuity to the initiatives of the VIVE UCN policy. The ENCUÉNTRATE, as a healthy pause instrument, did not suffer loss of succession over time because it was executed weekly and was always present in the university community. In addition, the trajectory of the VIVE UCN policy allowed theorising about its implementation stages: declaration, action and consolidation.

Another result that should be addressed is strengthening literacy in health promotion. A systematic literacy process in university health promotion could generate instances of appropriation of the VIVE UCN policy and differentiation between the policy and instruments such as ENCUÉNTRATE. Thus, the finding of the eclipse effect of ENCUÉNTRATE over the VIVE UCN policy could be addressed by various actions, such as literacy in basic concepts and health promotion (Xiangyang et al. 2003); promotion in fairs, which approaches and educates different audiences (Barranca-Enríquez et al. 2020), and the implementation of the policy in university curricula (Coffey and Coufopoulos 2010).

The perception of disparity between the more significant number of healthy activities for students compared to those offered to professors and administrative/technical staff is another finding of this study. This situation must be reviewed to reinforce future action plans that equitably integrate all university members (Khorsandi et al. 2022). Corrective actions considered in implementing health promotion strategies would allow equity and sustainability to be achieved as intrinsic values of health promotion (Suárez-Reyes et al. 2021).

The convening of the identity seal of the university community (Faria et al. 2024; Seeber et al. 2014) and the training of leaders in health promotion (Dooris et al. 2021) have emerged as a window of opportunity to address issues of sustainability of health promotion policy. These elements are recognised as healthy assets (Pérez-Wilson et al. 2022) and would facilitate the implementation of health promotion in universities. On the basis of the results, the construction of social capital within the university was conceived as a strength. Although it was not declared in the initial VIVE UCN policy, the dimension of social capital today emerges as a critical aspect of well-being and a healthier life (Farrier et al. 2017). For its part, the training of leaders should soon build bridges between the current policy situation and its desired state (Yao et al. 2021), as well as an organisational culture focused on health (Sedghi et al. 2023). In this way, the VIVE UCN policy should be consolidated as an organisational asset of the university to ensure that health and wellness are not relegated to marginal aspects of the mission and organisation of the institution (M. Reis et al. 2018). The way to manage the well-being of the educational community should be through health promotion indicators (Buijs 2009).

The results showed low participation of the professors in the VIVE UCN policy and ENCUÉNTRATE, which would be caused by an individual construction around the 'value of time' and the need to meet high academic standards. Therefore, any element that generates an imbalance between the dimensions of time and scientific productivity lacks relevance and interest among professors. This finding establishes notable differences with (Suárez-Reyes and Van DenBroucke 2023), where the most significant resistance to participate in healthy initiatives was given by the group of students and administrative/technical staff, which was explained by their lower participation in the decision-making process. The present study recognises student participation in the policy and healthy pauses but also recognises barriers, such as the high academic load among students who prioritised curricular issues over health and wellness actions. From the perspective of professors, the tendency was also to prioritise academic tasks, which ended up deteriorating their

health (Teixeira et al. 2020), ignoring significant findings that demonstrate the direct relationship between performance and productivity with participation in health promotion plans implemented in universities (Dooris et al. 2019).

On the basis of the perception of the university community, other aspects that need to be addressed for the consolidation of the VIVE UCN policy include the generation of instances of participation with professors (Suárez-Reyes and Van DenBroucke 2016) and empowerment of the social and collective dimensions (Pérez-Wilson et al. 2022), providing political and financial institutional support as a critical factor for health promotion strategies in universities (Suárez-Reyes et al. 2021), and promoting outdoor activities and connection with nature, as they have proven to be beneficial for physical and mental well-being (Farrier et al. 2017), given the intricate link between people and the settings (PAHO 2022).

4.1 | Study Limitations and Future Research Challenges

The study has some limitations. First, the sample design only included crucial actors linked to the VIVE UCN policy, potentially impacting the results. Second, the technique did not allow for participatory conversation or contrast information among participants (Acocella 2012). Finally, the research used the case study strategy, the critical criterion being a bounded system analysed in depth (Tomaszewski et al. 2020).

New lines of research have emerged from this study. From a methodological point of view, generating instances of participatory evaluations (Ingram et al. 2012) that bring together different stakeholders (Nitsch et al. 2013) is crucial. This would generate a deeper understanding of the policy and concept of health promotion. Although formal training in health promotion does not always produce the expected results (Morales-Torres et al. 2024), it would allow the ambiguous and broad term 'Healthy Universities' to be addressed (Innstrand and Christensen 2018). In thematic terms, it is necessary to deepen elements such as the dimension of student leadership (Reis et al. 2021), explore the provision of decompression spaces for mental and emotional health (Mowbray et al. 2006), disentangle if the educational experiences redistribute the welfare (Jongbloed and Pullman 2016), link the policy to wellness indicators that are evaluated over time, relate university health promotion initiatives to the SDG (Novieastari et al. 2022), harmonise the relationship between productivity and time among professors (Heng et al. 2020), implement situated and long-term educational experiences (e.g., Cortés et al. 2024) related to health promotion connects in theoretical and practical fields and explore potential relationships between space (campus) and the subjective and social well-being of community members from the perspective of ecosystem services (e.g., Nguyen and Truong 2024).

5 | Conclusion

This study explores the perceptions of implementing the VIVE UCN policy, the first university health promotion policy in Chile and one of the first in Iberoamerica. The study design

enables the replication of the evaluation of health policy deployment in universities and broader educational settings and establishes that the adoption and acceptance of health promotion initiatives in educational settings facilitate the definition of institutional health promotion policies. Students, professors and administrative/technical staff had differing views; overall, the policy was positively valued, highlighting past, present and future perceptions. The results indicate a participative formulation of the policy, current challenges with disparate collective participation and institutional efforts to strengthen future projections for its consolidation. Other findings show that the ENCÚENTRATE healthy pause significantly overshadows the VIVE UCN policy, likely due to turnover among professors, administrative/technical staff and rotating university students.

Dissimilar levels of participation in the university community are caused by the high academic load among students, despite the generous institutional offer of activities that promote health and wellness. The VIVE UCN policy fosters a robust institutional identity among administrative and technical staff, who actively participate in collective wellness activities and self-care practices during the ENCÚENTRATE healthy break. They also expressed concern about potentially losing these healthy activities due to certain practices that strayed from the main objective of VIVE UCN. Professors participate in strategic and planning activities as needed. However, their daily involvement in healthy activities is limited due to their focus on academic and research pursuits to meet performance indicators.

Other findings show the eclipse effect of the ENCÚENTRATE healthy pause on the VIVE UCN policy. The probable causes of this situation point to the lack of knowledge of the values and sense of policy generated by the loss of succession, which occurs when professors and administrative/technical staff are renewed and students' universities rotate after graduation.

For future projections, facing a consolidation stage, a window of opportunity emerges to incorporate healthy indicators in institutional management and to take advantage of the university community's sense of identity to form leaders in health promotion and wellness. This will improve staff and professors' health and wellness promotion initiatives, encouraging active participation.

Author Contributions

Mónica Castillo-Rosales: conceptualization, data curation, formal analysis, investigation, resources, supervision, visualisation, writing – original draft, writing – review and editing. **Angelo Araya-Piñones:** conceptualization, formal analysis, investigation, methodology, software, writing – original draft, writing – review and editing. **Irma Brito:** formal analysis, investigation, writing – original draft. **Marisa Afonso de Andrade Brunherotti:** formal analysis, investigation, writing – original draft. **Verónica Alfaro-Navarro:** formal analysis, investigation, writing – original draft. **Nicolas Van Niekerk Bakit:** formal analysis, investigation, resources, writing – original draft.

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Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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